34. DAY 2 QUESTIONS

DURATION : 08:08

STUDY SHEET

COURSE SUMMARY

This video explores the importance of cerebral movement in cranial osteopathy, highlighting how the body, as an intelligent entity, develops autonomously. Key concepts include the link between the viscerocranium, heart, diaphragm, and immune system, showing that dysfunctions at one level can affect other systems. The interconnection between the diaphragm and organic function is emphasised, while also stressing the importance of working on diaphragm dynamics to improve patients' quality of life. Practical exercises and theoretical notions support learning on how to balance interconnected bodily structures.

34. DAY 2 QUESTIONS: BIODYNAMIC EMBRYOLOGY AND THE HUMAN BODY

BRAIN MOVEMENT: FOUNDATION OF CRANIAL OSTEOPATHY

The **movement of brain development** is fundamental and provides profound meaning in **cranial osteopathy**. The human body is intrinsically intelligent and knows the pathways of its own development.

Our role is not to re-educate the brain, but to learn to listen to it. It will instruct and guide us.

We will learn to "do the **brain's Tai Chi**", observing that the **base of the skull** is intimately connected to the brain. The **ventricular system** and the **deep flow of the brain** depend on its dynamics.

Similarly, the entire **membranous system** (tension membranes, falx cerebri, tentorium cerebelli, falx cerebelli, dural membranes, parietal, visceral) is dependent on cerebral dynamics. This compartmentalisation of the brain, from the dense structure of the cranial base to the vault, is a reflection of these dynamics.

THE VISCEROCRANIUM AND ORGANIC CONNECTIONS

We will study the **viscerocranium**, starting with the **septum transversum**. The latter is a tissue integration including the heart, the diaphragm, and extending with the **falciform ligament**.

This structure reveals an early functional integration between the heart and the liver. It continues via the **foramen of Winslow** to form the **greater omentum**.

IMPLICIT RELATIONSHIPS

This means there is a fundamental relationship between:

- **The heart**
- **The diaphragm**
- **The brain**
- **The immune system**

Destabilisation at the level of the heart or the base of the skull can therefore impact the immune system. The superior immune system, **Waldeyer-Heiner's ring** (tonsils), is an example of this.

IMPACT ON THE ENT SPHERE

Dysfunction at this level can have significant repercussions. For example, many children suffering from **ENT** problems present, at a molecular level, the same **immunoglobulin** (type 1) as that found at the caecal level.

Destabilisation of the **peritoneum** can thus affect the entire ENT sphere. In practice, rebalancing the ENT sphere often requires rebalancing the peritoneum, particularly at the caecal level, which contains lymphatic tissues identical to those of the tonsils. It is interesting to note that in the past, tonsillectomies and appendectomies were often performed synchronously.

THE OESOPHAGUS AND ITS INTERCONNECTIONS

The oesophageal relationship is also crucial. The **oesophageal mesenchyme** can disrupt the function of the diaphragm.

The stomach and oesophagus are suspended up to the base of the skull via the **carotid foramina**, the **pterygoid plexuses**, and the **pterygoids**. Any destabilisation of the base of the skull can therefore alter **cardio-tuberosity** function and, consequently, diaphragmatic movement, which is essential to our physiology.

THE MESOBLASTIC WALL AND THE DIAPHRAGM

The **lateral mesoblastic wall** (lateral wall) is also very important. The **costal grid** is entirely dependent on the proper functioning of the diaphragm.

Consequently, the entire dorsal area is connected to these dynamics. Furthermore, we have noted a junction with the **kidney**, the **psoas**, and the **duodenum**, highlighting the overall integrity of the system.

THE IMPORTANCE OF THE DIAPHRAGM

We must learn to work the diaphragm with more subtlety, whether remotely or not. By expanding this diaphragm, the patient's quality of life is significantly improved.

When exercising the diaphragm, an essential reference point is its **fulcrum**. It is possible to suspend this fulcrum in the dynamic stillness of space.

It's like a door that opens and closes, with a hinge, a **"Hitchpoint"** or fulcrum. This starting point may seem mobile, but on deeper investigation, it becomes a fixed point, your reference, but always suspended in the dynamic stillness of the system.

THE HEART, THE PERITONEUM, AND EMBRYONIC MOVEMENTS

A question arises concerning the **meso-oesophagus** and the **vena cava**. The latter is very close to the heart, almost on the same axis, and not far from the liver.

The first great movement of the **notochord** and the second, circular, movement of the **amniotic cavity** involve a visceral centre that we identify with the heart and the peritoneum.

THE IMAGE OF COILING

The image is one of **coiling** around the **cardio-pleuro-peritoneal** system, like a joint. The **neural tube** develops strongly forwards, drawing the heart, but it also grows backwards, bringing the allantois through an anterior restriction field.

This means there is a closing movement of the body by the **embryonic pedicle**. It is at this moment that the allantois and the heart integrate, becoming a fulcrum.

THE ORIGIN OF THE SEPTUM TRANSVERSUM

At this stage, two elements are crucial: the heart pushing on the southern part and the upper part of the **vitelline vesicle**. This push orientates and organises the **septum transversum**.

Its tissue origin was already present (the cells were there), but this encounter between the brain and the heart, and the heart and the vitelline vesicle, provides the impetus for the creation of the septum transversum. The heart thus acts as a joint between the brain and the diaphragm.